

IN THIS ISSUE

- 2 Message from the Director
- 3 Can We Talk?
- 8 ACS Nutrition/Activity Guidelines
- 10 New Board Member
- 10 In Memoriam
- 11 Events Calendar

Cancer Screening Made Personal

There's no question that cancer screening tests, such as mammography and PSA testing, are capable of finding cancers in their earlier, more curable stages. But years of evaluation and hundreds of thousands of patients later have taught healthcare professionals something else: Cancer screening is not "one size fits all."

That's precisely why, as the NYU Cancer Institute expands its cancer screening facilities, the screening exams recommended for each patient are being customized — taking into account the patient's personal medical history, family history, and other risk factors, such as lifestyle, known to affect cancer risk (such as smoking and alcohol). "There are clear benefits to cancer screening, but it has to be tailored to each patient's personal situation," explains William L. Carroll, MD, NYU Cancer Institute Director.

Which cancer screening tests do you need? Here are some guidelines for and updates on the most common exams:



Low-dose CT scanning is now offered as a screening test for lung cancer in people at increased risk for this disease.

Breast Cancer Screening

Mammography has been shown to save lives. Studies show that breast cancer death rates are 25 percent lower among women who have regular mammography screening. Moreover, treatment for early-stage breast cancer is typically less intense and easier to tolerate than therapy for more advanced disease.

continued on page 4



Nurse Katy Donahue is one of the coordinators of the *Thriving!* program for survivors of childhood cancer.

Thriving! Program Supports Survivorship for Children During and After Cancer Treatment

Tremendous strides have been made in the treatment of pediatric cancers, and more children are surviving than ever before. But success sometimes comes at a cost. The Children's Cancer Survivor Study (CCSS), an analysis of more than 10,000 adults who were diagnosed with cancer during childhood or adolescence between 1970 and 1986, has reported that more than 60 percent of survivors experienced chronic health problems later in life. These "late effects" included heart disease, lung scar-

ring, infertility, and second cancers due to radiation therapy or chemotherapy.

The good news is that the CCSS findings have been used to tailor today's pediatric cancer treatments to reduce the risk of such late effects without compromising effectiveness. For example, radiation therapy to the brain — once a standard treatment for acute lymphoblastic leukemia (ALL) to treat possible cancer spread to the nervous system — is now

continued on page 6

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Message from the Director

Today there are more people surviving cancer than ever before, thanks to more effective treatments and earlier diagnosis through screening. At the NYU Cancer Institute, we celebrate these improvements in cancer survival. But we also recognize that it is not enough just to survive. It is also important to live.

That's why we've implemented a number of initiatives to enhance life not only during cancer treatment, but after therapy as well. You can read about one of them in this issue —

the *Thriving!* program at the Stephen D. Hassenfeld Children's Center for Cancer and Blood Disorders at NYU Langone Medical Center. This program formally unifies and expands upon our efforts to address all aspects of the development of each child and adolescent we treat, from the first day of therapy until long after treatment has been completed. I'd like to thank Abbe and Brian Walter for their support of this vital program.

In adults, detecting cancer early increases the chance of curing it. The NYU Cancer Institute has always provided a wide range of cancer screening services, and has expanded them further. Today we know that each person's risk of cancer is a unique blend of lifestyle and genetic risk factors, and we take this into account to customize each patient's care. You can read about screening on page 1.

For people being treated for cancer and for anyone who sees a doctor, your ability to take care of your health depends a great deal on your ability to understand what your healthcare team is telling you. Patients who clearly understand what their doctors are telling them are more likely to comply with the care prescribed for them, and they have the best chance of a good outcome. To learn how to improve your communication with your doctors, see page 3.

Of course, the best approach is to prevent cancer from developing at all. There are steps people can take to reduce their risk of the disease, such as eating a healthy diet and maintaining a healthy weight. In this issue, we provide simple recommendations to help readers adhere to the new *American Cancer Society Guidelines on Nutrition and Physical Activity for Cancer Prevention*.

Our focus on wellness not only reduces the burden of cancer on individuals, but on families and society as well. You can start today by moving more and eating healthier. And with the warm weather upon us, there's no better time to begin than now. Be well!

William L. Carroll, MD



The Julie and Edward J. Minskoff Professor of Pediatrics
Professor of Pathology
Director, NYU Cancer Institute



Can We Talk?

Communicating with Your Healthcare Team

How well do you really understand what your doctor tells you? Consider these statistics:

► A study in the *Journal of the Royal Society of Medicine* found that patients forget up to 80 percent of what their doctors tell them as soon as they leave the office, and nearly half of what they do remember is recalled incorrectly.

► A study of two public hospitals in Atlanta showed that 26 percent of respondents could not understand when their next appointment was scheduled, and 42 percent did not understand instructions to “take medication on an empty stomach.”

► The U.S. Department of Education reports that 36 percent of adults have basic or below-basic skills for understanding health-related materials.

And that’s just the beginning. Hearing you have cancer can elevate your level of anxiety to a point where you’re not fully able to receive the information that your doctor tells you afterward. Chemotherapy can hamper your memory and thought processes. And doctors are more rushed today than ever before. So how do you optimize your communication with your healthcare team?

“There may not be adequate time for you to think of all your questions while in the office,” maintains Deborah Axelrod, MD, Associate Professor of Surgery and Medical Director of Community Outreach and Education at the NYUCI. “So it’s important for patients to come prepared with their questions, and take steps to ensure that they understand their care and what the healthcare team is telling them before they leave the doctor’s office.”

Understanding what your healthcare team tells you isn’t just a matter of making you feel empowered; it can actually translate to better outcomes. For people with cancer or those at increased risk for the



Learning to communicate well with your healthcare team can translate to better outcomes.

disease, a low ability to understand their health care can result in absent or less frequent screening, poor compliance with treatment, and inadequate follow-up care.

So what can you do? Our experts recommend a collaborative relationship in which you share decision-making with your doctor. Here are tips on how to achieve that goal:

- 1. Come to your appointments prepared.** Write down any questions or concerns you may have, and prioritize them so you ask the most important questions first. Make a list of all the medications you are taking, as well as your other doctors. Bring paper and pen to take notes.
- 2. Consider bringing someone with you to appointments.** Someone else may hear things you do not. And that person can serve as a source of comfort, relieving your anxiety so you are better able to hear what your doctor is telling you.
- 3. Ask for explanations when you don’t understand.** If your doctor is using terms you do not know or is speaking too quickly, ask him or her to slow down and explain the information more clearly in terms you can comprehend.
- 4. Don’t be afraid to ask for a second opinion.** Some people feel they may insult the doctor by stating they want a second opinion. But you are entitled to it. Your doctor

will understand.

5. Be completely honest. Answering your doctor’s questions truthfully is important to ensure that you receive the most effective care available.

6. Ask your doctor for the best way to contact him or her with follow-up questions. Most prefer the phone, and some are okay with e-mail.

7. Speak up if your doctor seems too busy. Ask him or her to slow down. If you feel uncomfortable asking, you can say something like “You seem very busy today.” Don’t feel a need to apologize for taking up their time.

8. Raise any concerns about practical and financial matters. If you are concerned about the cost of your care, your insurance coverage, or your ability to come for treatments, bring up these issues with your healthcare team.

9. Set the stage. Some patients feel more confident talking with their doctors when wearing street clothes instead of a gown. Speaking at eye level can also be helpful. Turn off your cell phone and any other lines of communication.

10. Consider a support group. Speaking with other patients who share the same experience as yours can provide insights about your care and how to communicate with your healthcare team. ■

For women at average risk of breast cancer, the American Cancer Society recommends an annual screening mammogram starting at age 40. But according to the U.S. Preventive Services Task Force (USPSTF) guidelines, having a mammogram every two years between the ages of 50 and 74 is sufficient. The USPSTF also states that mammography in women younger than 50 should be based on medical and family history and personal values, and notes that evidence supporting screening mammography in women age 75 and over is “insufficient.”

Other professional groups are closer to the ACS guidelines, with some noting that

should assess the biology of her tumor and other aspects of her health to tailor a treatment plan that is as effective as possible while reducing the impact on a woman’s life.

To learn when and how often you should get a mammogram, speak with your doctor.

PSA Testing for Prostate Cancer

PSA testing has helped increase the number of early-stage prostate cancers that have been detected, and men treated for cancers that are detected with screening before they have spread beyond the prostate have an excellent chance of long-term survival.

in not being overly aggressive in searching for small cancers in the elderly. In fact, prostate cancer is so common in elderly men that many die *with* the disease, rather than *from* it.

The most important current controversy is whether or not routine PSA testing should be used to screen for prostate cancer. Current studies suggest that while there may be an overall reduction in the number of prostate cancer deaths, the reduction in death achieved through screening may not be enough to offset the significant side effects and costs incurred by biopsies and treatment. This is an important question, because while most prostate biopsies are harmless, some men develop complications such as infections. And treatment for prostate cancer can raise the risk of impotence, incontinence, and rectal problems, depending on the therapy, so it is important to know who really needs to be treated and who can be monitored.

The American Cancer Society has eased its guidelines for PSA testing over the years, stating that asymptomatic men age 50 and over with at least a 10-year life expectancy should have an opportunity to make an informed decision with their healthcare providers about screening for prostate cancer “after receiving information about the uncertainties, risks, and potential benefits associated with screening. Prostate cancer screening should not occur without an informed decision-making process.” The USPSTF has backed away from recommending PSA screening at all, supporting its use only in men who have symptoms — noting that the benefits of PSA testing are not sufficient to offset the cost of prostate cancer therapy and its side effects.

“While many cancers of the prostate are not lethal, unfortunately many are and can be effectively treated through early detection. So rather than avoiding PSA testing altogether, we need to do better at deciding who should be tested and how to interpret the test results in individual patients,” says Samir Taneja, MD, the James



NYU Langone Medical Center is designated as a “Breast Imaging Center of Excellence” by the American College of Radiology.

women ages 40 to 49 should have mammography every one to two years. The differences center on the benefits (detecting breast cancer early) versus harms (performing biopsies for lesions that turn out to be benign) of the test and its cost-effectiveness (such as how many lives it saves).

“Many women feel like they are receiving mixed messages,” says Freya Schnabel, MD, Professor of Surgery and Director of Breast Surgery. “Some physicians have also changed the recommendations they give to their patients. As a result, fewer women are being screened.”

Dr. Schnabel notes that while overdiagnosis may not harm a patient, overtreatment might. When a woman is diagnosed with breast cancer, her healthcare team

That’s the good news.

But for all its benefits, the PSA test — which detects a protein in the blood called prostate-specific antigen (PSA) that rises with prostate cancer growth — has several limitations as well. For one thing, it is not highly specific to prostate cancer, rising in response to noncancerous conditions such as benign prostate enlargement or infection. And most importantly, while it is very sensitive for detecting cancer, many small cancers that are detected are not likely to cause death (particularly when a biopsy is performed in a man with a low PSA level, or the patient is older and more likely to die of other causes).

Because prostate cancer is so common with aging, physicians must exert caution

M. Neissa and Janet Riha Neissa Professor of Urologic Oncology and Director of Urologic Oncology. “The fundamental question should be, ‘When do we treat, and when do we observe?’”

The NYU Cancer Institute supports a “risk-stratified” approach to prostate cancer screening, considering a man’s PSA level in context with other risk factors for prostate cancer, like obesity, diabetes, ethnicity (African Americans have an elevated risk), and family history. In some men with an elevated PSA and those with other risk factors for prostate cancer, doctors are coupling the test results with the findings of imaging tests, such as magnetic resonance imaging, to better assess the cancer and determine what additional care is necessary. They may also conduct imaging in combination with a urine test for a protein called PCA3, which is more specific than PSA (more likely to signal prostate cancer over noncancerous conditions).

“We’d like to use imaging tests and biomarkers, such as urine PCA3, to decide who should have a prostate biopsy and how it should be performed,” says Dr. Taneja. “Our aim is to make screening and assessment an individualized and beneficial process, rather than to simply abandon it.”

CT Screening for Lung Cancer in Smokers

NYU Langone Medical Center has long been a leader in research to evaluate the effectiveness of low-dose CT scanning to screen smokers and others at increased risk for lung cancer through its participation in the National Cancer Institute Early Detection Research Network. In the summer of 2011, the results of the National Lung Cancer Screening Trial reported what doctors have been hoping for a long time: that low-dose CT screening can actually save lives, reducing the death rate from lung cancer by 20 percent among the more than 53,000 individuals who participated, and was far superior to chest x-ray.

“We now have proof that patients



The newly opened Center for Women's Imaging provides screening mammography services (including same-day results on weekdays) and convenient hours in a warm and inviting environment.

whose lung cancer is detected early by CT scanning have a better chance of survival than if the cancer remained undetected,” says Harvey Pass, MD, the Stephen E. Banner Professor of Thoracic Oncology, Chief of Thoracic Oncology, and Chief of the Division of Thoracic Surgery.

People at increased risk for lung cancer can now get the test at NYU. The exam is typically not covered by insurance, and costs \$350. Patients require a prescription from a doctor and fill out a health questionnaire before having the exam. The radiation dose from the scan is very low — about

one-fifth the dose of a standard CT scan. Patients whose scans are negative will be told to come back a year later. Those with suspicious findings on the CT scan may have another scan in six months or be referred for other follow-up, depending on the results.

People at high risk for lung cancer in whom CT screening may be recommended include those age 50 and over with a 20-pack-year smoking history and patients with other lung cancer risk factors, such as occupational exposure to asbestos or other cancer-causing substances.

Other Screening Tests

The American Cancer Society also recommends screening for colorectal cancer starting at age 50 for men and women, and periodic cervical cancer screening with a Pap test for women. These services are also available through the NYU Cancer Institute. For more details on the guidelines, visit www.cancer.org. ■

NYUCI Expands Screening Centers

The NYU Cancer Institute is expanding its screening facilities. These include:

A New Center for Women's Imaging

Screening mammography services, previously offered at the NYU Clinical Cancer Center, recently moved around the corner to the new NYU Langone Center for Women's Imaging at 221 Lexington Avenue at 33rd Street. The center features:

- New dedicated facility designed for comfort and personal attention
- Same-day appointments and convenient weekday, evening, and Saturday hours
- New real-time reading option, with results provided at time of visit (on weekdays)
- Ability to book next appointment before you leave the center
- Participation in most insurance plans
- Genetic counseling and evaluation for individuals at increased risk for cancer

For an appointment at the Center for Women's Imaging, call 212-731-5002.

Screening mammography will also be available at the Joan H. Tisch Center for Women's Health at 207 East 84th Street in New York City. To make an appointment, call 646-754-3300.

CT Screening for Lung Cancer

Low-dose CT screening is now available at NYU Langone's facility at the Rivergate building on 401 East 34th Street. To make an appointment, call 855-NYU-LUNG. For more information, visit <http://lung-cancer-screening.med.nyu.edu>.

rarely used because it was found to increase the risk of developmental delays in the child; fortunately, chemotherapy has replaced the need for radiation. Additionally, doctors are more aware that individuals treated for cancer when they were young require special follow-up and monitoring as adults.

“As we increase the number of children with cancer whom we cure, the

aspects of a child’s or adolescent’s physical, cognitive, and emotional development from the first day of care until long after treatment ends. The program will draw upon the unique strengths of NYU Langone Medical Center’s comprehensive services for pediatric cancer patients. (See page 7 for more information.)

“Childhood is about social and emotional growth and education — crucial



The staff of the *Thriving!* program provide specialized care to nurture and heal all aspects of a child’s or adolescent’s development during and after cancer treatment.

number of adults who are childhood cancer survivors continues to increase,” says Linda Granowetter, MD, Director of the Stephen D. Hassenfeld Children’s Center for Cancer and Blood Disorders at NYU Langone Medical Center.

To address the survivorship needs of Hassenfeld Center patients, a program called *Thriving!* was created. The long-term follow-up program was established with a generous gift from Abbe and Brian Walter, whose son, Ben, was successfully treated at the Hassenfeld Center starting in 2003, when he was diagnosed with ALL at 23 months of age. William L. Carroll, MD, NYU Cancer Institute Director, directed Ben’s care. The Making Headway and Tom Coughlin Jay Fund foundations have provided additional funding.

The *Thriving!* program will provide specialized care to nurture and heal all

areas affected by cancer,” explains David Salsberg, PsyD, who is overseeing the psychological and neuropsychological components of the *Thriving!* program with Hassenfeld psychologist Laura Tagliareni, PhD, and Preeti Saigal, PhD, of the Rusk Pediatric Psychology Service. “How we prevent, monitor, and address issues that can arise will positively impact a child’s life forever. Now that these children are surviving, it is our charge to assure that they will thrive.”

Recognizing the Need

When Ben Walter was diagnosed with ALL, he was a spirited toddler who loved putting together puzzles and building with blocks. Three years and two months of chemotherapy cured him of the cancer. Today he is a 10-year-old fourth grader in New Canaan, Connecticut, who excels in school.

After treatment, however, Ben’s parents noticed subtle but significant changes in his ability to think of words and in his processing speed, as well as difficulty with fine motor skills (holding crayons or pencils, building blocks, etc.) and gross motor skills (such as coordination, balance, strength, and flexibility). As a clinical psychologist, Abbe Walter realized that her son needed annual neuropsychological evaluations, occupational and physical therapy evaluations, and ongoing rehabilitation and treatment in these areas from professionals familiar with pediatric cancer — a challenging task that they coordinated themselves. Services were available, they found, but not in a formal, centralized way.

Abbe and Brian Walter also wondered, what about parents who don’t have the educational background or resources that they have? “As we thought about the needs of our son, we thought about survivorship issues for all children,” says Mr. Walter, a partner at a boutique investment bank. “What would people who don’t have Abbe’s knowledge do? We place so much focus on curing a child of cancer, as we should, but life after cancer is important as well.”

“Dr. Carroll and the whole Hassenfeld staff are amazing, and we always wanted to find a way give back to NYU,” adds Dr. Walter. “It isn’t over when treatment is over. We decided to support the *Thriving!* program to help address the effects of treatment and the transition back to school, and to help survivors learn how to be advocates for themselves.”

The Hassenfeld Center has always provided survivorship services for patients and their families, but the *Thriving!* program unifies them in a formal initiative. “We are ensuring that all of our survivors have the resources they need to have an optimal quality of life,” explains Katy Donahue, RN, CPHON, Nurse Coordinator at the Hassenfeld Center, who, with Erin Hartnett, DNP, APRN-BC, CPNP, is coordinating the

Thriving! program. “To reduce the possibility of late effects, we are tailoring treatment from day one, with an overall focus on wellness.”

Ms. Donahue has first-hand experience in this area, and not just as a nurse. She is a 30-year survivor of osteosarcoma, diagnosed with bone cancer in her left hip at age three and successfully treated with chemotherapy and limb-sparing surgery.



Brian and Abbe Walter made a generous donation to support the *Thriving!* program for pediatric cancer survivors.

When she was 24, a doctor asked her if she ever had an echocardiogram (an ultrasound of the heart). She was unaware her cancer treatment made her a candidate for future heart disease, and that the typical risk factors for the illness — such as smoking — could adversely affect her even more so than her peers who were not treated for cancer. “It’s important for survivors of pediatric cancers to know what they need to do to stay healthy as adults,” Ms. Donahue says.

The *Thriving!* program will have a positive, profound, and sustained impact on patients as they grow from childhood and adolescence into adulthood and beyond. Says Dr. Carroll, “This program is designed to reduce the burden of cancer on patients and their families by promoting wellness and improving quality of life from day one until long after treatment has ended.” ■

Thriving! Program Services: Care and Aftercare

From the first day a child is diagnosed with cancer, the Hassenfeld Center provides the care, information, and resources young patients need to ameliorate or prevent later complications. The Hassenfeld team employs innovative interventions and services such as nutritional counseling for lifelong wellness; early physical and occupational therapy for patients at risk for nerve problems or disability; early neuropsychological evaluations for patients at risk of cognitive impairment; and early intervention and school programs.

After treatment is complete, survivors are cared for and tracked for at least ten years. Each survivor meets with members of his/her individualized *Thriving!* program team, which includes the patient’s personal physician or nurse practitioner, a psychologist, social worker, nutritionist, recreation therapist, a medical librarian, a teacher, a fertility specialist, and a physical and occupational therapist as needed. Moreover, patients and their families can meet with all members of the team on the same day, to prevent the need for repeat visits to the center.

Because the Hassenfeld Center draws on the unique strengths of NYU Langone Medical Center’s clinical, educational, and psychosocial programs, the depth of services offered at the *Thriving!* program is expansive, and includes:

- ▶ Psychological and neuropsychological screening and services to assess and address the emotional, social, psychological, and developmental needs of each child. A particular strength of the program is support for adolescents and young adults, who may feel like they “lost” their teenage years because they were being treated for cancer. A tremendous amount of resources are also put into solidifying ongoing learning, remedial, and treatment programs for these children, in and out of school.
- ▶ Educational support, with a teacher working with children during therapy and after treatment, to ensure they receive the support they need at school or with a tutor.
- ▶ Resource support, with a medical librarian who helps families access resources about the patient’s follow-up needs.
- ▶ Recreation therapy and nutritional counseling, with a focus on helping patients achieve and maintain a healthy lifestyle through physical activity and a healthy diet.
- ▶ Physical and occupational therapy to improve gross motor and fine motor skills that may be affected by cancer treatment. Patients benefit from a unique relationship between the Hassenfeld Center and NYU Langone’s Rusk Institute of Rehabilitation Medicine.
- ▶ Support groups and other events tailored to the needs of children and adolescents who have completed treatment, and their families.
- ▶ Fertility services, since some cancer treatments raise the risk of infertility. NYU Langone Medical Center is home to one of the most renowned infertility programs in the country. “We have the resources to offer the most cutting-edge methods of fertility preservation, such as egg freezing, to our patients,” notes Dr. Granowetter.
- ▶ A research component of the *Thriving!* program aims to better understand late effects of cancer care, such as obesity and infertility, and to refine the long-term care guidelines for survivors.

Each patient also receives a binder called the Passport to Care, which includes a detailed treatment history and follow-up guidelines. This binder contains details about which drugs they received and at what doses, what kind of surgery they had and when, and how much radiation therapy they received, if that was part of their care. “This information is critical to ensure continuity of care. We encourage and empower patients to become strong advocates for their self-care,” says Ms. Donahue.

You Can Reduce Your Risk of Cancer...Here's How!

Many of us know that smoking is a leading cause of cancer. Tobacco use is responsible for a third of all cancers in the United States. But did you know that just as many cancers are due to poor diet and being overweight?

According to the American Cancer Society (ACS), for most Americans who do not use tobacco, the most important cancer risk factors that can be modified are body weight, diet, and physical activity. Getting regular physical activity and eating a nutritious diet that is low in fat and high in fruits, vegetables, and whole grains can also reduce your risk of diabetes and heart disease.

"We all know fruits and vegetables are good for us," says Niyati Parekh, PhD, RD, Assistant Professor of Nutrition and Public Health at New York University's Steinhardt School and a member of the NYU Cancer Institute. "It's just a matter of how driven you are."

Knowledge is important, but motiva-

tion is key, Dr. Parekh notes. Diet and exercise guidelines recommend regular physical activity and five to nine servings of fruits and vegetables daily. So how do you work more activity into your busy week? Or build more fruits and veggies into your diet? Or maintain a healthy weight when you're confronted with pastries and other treats in your office kitchen? We'll give you some tips to start you on your way.

The Facts and the Evidence

The ACS updates its *Guidelines on Nutrition and Physical Activity for Cancer Prevention* about every five years. The latest guidelines advise people to get at least 150 minutes of moderate-intensity exercise (like brisk walking) or 75 minutes of vigorous exercise (like running) every week (an hour a day of activity for kids); to limit the amount of time spent sitting; to limit intake of processed and red meats; to eat 2½ cups of fruits and vegetables each day; to incorporate whole grains into the diet; and to limit

alcohol intake. (See page 9 for specific guidelines.) The ACS recommendations are based on epidemiological studies — research done in large populations to identify risk factors for cancer and quantify their impact. For example, studies have found that:

- Being overweight or obese raises the risk of cancers of the breast, colon/rectum, uterus, esophagus, kidney, and pancreas, and may increase the risk of lymphoma, multiple myeloma, and gallbladder, liver, cervix, ovarian, and aggressive prostate cancers.
- Physical activity may reduce the risk of breast, colorectal, uterine, and advanced prostate cancers.
- Higher vegetable and fruit intake reduces cancer risk.

What You Can Do Today

Even if you've never walked around the block or cooked a fresh vegetable, it's not too late to start making changes to your diet and exercise routine. Here's how to get started:

- 1. Visit your local farmers' market.** See what produce is in season. Try products you've never had before. "Increasing fruit and veggie intake doesn't have to be boring. Mix the types and colors of your produce to keep it interesting," says Dr. Parekh.
- 2. Add fruit to your breakfast.** One way to start eating more fruits and veggies is to add one serving of fruit at breakfast, like sliced banana on your cereal or orange slices alongside your toast. Gradually add another serving at lunch, and eventually step up your intake at dinner.



Niyati Parekh, PhD, RD, provides simple guidance on diet and exercise to reduce your risk of cancer.



PLATE MAKEOVER

1050 to 560 calories!

980 to 550 calories!

With a "Plate Makeover," you can still have the foods you love, prepared in healthier ways.

3. Don't shy away from canned or frozen.

There's no harm in eating fruits and vegetables that are canned or frozen (as long as they're not swimming in sugar). They still contain healthy nutrients and fiber and are an acceptable alternative to fresh produce.

4. Give your meals a makeover. You can still have the foods you love while eating a healthy diet. If you crave fish and chips, replace it with broiled fish and mashed potatoes instead. Use less cheese in your macaroni and cheese, and mix in some spinach for added color and nutrients.

5. Plan it, pack it. You don't have to eat out or get take-out at lunchtime. Take a few extra minutes before you go to work to make a sandwich and some healthy snacks, or pack leftovers to reheat at lunchtime.

6. Take extra steps...literally. Studies show that sedentary behavior — such as sitting — is an independent risk factor for cancer. “Even if you get your 150 minutes of moderate intensity exercise every week, if you're sitting around or sleeping the rest of the time, it's not good,” explains Dr. Parekh. Limit your time spent in front of a computer or television. If you are watching TV, get up and walk in place during commercials. If you sit in an office most of the day, get up frequently for small breaks.

7. Go the distance. Get off the subway or bus one stop earlier and walk the rest of the way, park farther away at the mall or supermarket, or take the stairs. Think of ways you can walk more during your day.

8. Register for a class. Sign up for a new activity at your gym, or slip into a yoga class at lunchtime. Take a class to learn how to cook healthy meals based on fruits, vegetables, beans, and whole grains.

“There's no doubt that many cancers have a clear link to obesity,” adds NYU Cancer Institute Director William L. Carroll, MD. “We need to change societal factors that contribute to obesity. Overcoming it is a challenge that needs to be started early in life and adopted by families.” ■



Here's what the American Cancer Society recommends to reduce your risk of cancer through diet and exercise:

Achieve and maintain a healthy weight throughout life.

- Be as lean as possible throughout life without being underweight.
- Avoid excess weight gain at all ages. For those who are overweight or obese, losing even a small amount of weight has health benefits and is a good place to start.
- Get regular physical activity and limit your intake of high-calorie foods and drinks to help maintain a healthy weight.

Be physically active.

- **Adults:** Get at least 150 minutes of moderate intensity or 75 minutes of vigorous intensity activity each week (or a combination of these), preferably spread throughout the week.
- **Children and teens:** Get at least 1 hour of moderate or vigorous intensity activity each day, with vigorous activity on at least 3 days each week.
- Limit sedentary behavior such as sitting, lying down, watching TV, and other forms of screen-based entertainment.
- Doing some physical activity above the usual activities, no matter what your level of activity, can have many health benefits.

Eat a healthy diet, with an emphasis on plant foods.

- Choose foods and drinks in amounts that help you get to and maintain a healthy weight.
- Limit how much processed meat and red meat you eat.
- Eat at least 2½ cups of vegetables and fruits each day.
- Choose whole grains instead of refined grain products.

If you drink alcohol, limit your intake.

- Drink no more than 1 drink per day for women or 2 per day for men.

Source: *American Cancer Society Guidelines on Nutrition and Physical Activity for Cancer Prevention, 2012*. For more specific information, visit www.cancer.org.



MyPlate was created by the U.S. Department of Agriculture to help people make healthy choices at every meal. This diagram shows the recommended portions of fruits, vegetables, grains, protein, and dairy products. To learn more about healthy eating and adding physical activity to your life, visit www.choosemyplate.gov. More information about healthy eating can also be found at www.eatright.org.

Constance McCatherin Silver Joins NYUCI Board



The NYU Cancer Institute has announced the appointment of Constance McCatherin Silver to its Board of Advisors.

Dr. Silver has been a member of the NYU Board of Trustees since 2003 and serves on the Academic Affairs, University Life, and Global Affairs Committees. In 2007, she and her husband, Martin, made a contribution to the NYU School of Social Work, which is now the NYU Silver School of Social Work.

Dr. Silver graduated from the NYU School of Social Work, earning a BS degree in social work in 1978 and an MSW degree in 1979. She also received a PhD from the Union Institute and University in 1983. Dr. Silver has been a practicing social worker

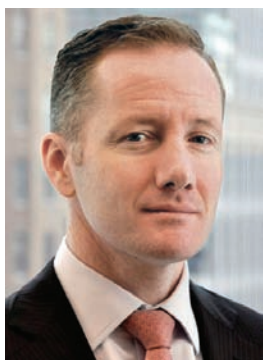
and psychoanalyst in New York City since the early 1980s.

She maintains affiliations with the Training Institute of Mental Health, Union Institute and University, and the Martin and Dr. Constance Silver Education Center (a Florida Humane Society facility), among other organizations. She was born in Maine and continues to have close ties with the University of Southern Maine and several waterfront associations.

The Silvers' primary residence is Indian Creek Island, Florida, where Dr. Silver serves as the Special Training Director for the local public service agency. The Silvers also spend time in New York City and Westport, Connecticut. In her spare time, she notes that "her love of art holds forth."

"When thinking of the important goals in society, I put education and health improvement on the top of my list," Dr. Silver says, describing her decision to join the NYUCI Board. "I believe that, in one fell swoop, I am fortunate enough to be able to take a swipe at each one." ■

Remembering Jeremy Hill



The NYU Cancer Institute mourns the death of Jeremy Hill, who was a member of the NYUCI Advisory Board since 2010. He died in March at age 43 after a courageous fight against melanoma.

Mr. Hill was a Managing Director and Co-Head of Global Equity Finance at J.P.Morgan, a unit within the firm's Prime Brokerage business. Prior to this role, he was a Senior Managing Director and Global Head of Equity Finance at Bear Stearns. A native of New Zealand, Mr. Hill held a BCom from the University of Auckland

and an MBA from the University of Chicago.

He also served on the Board of Directors of the SAMFund, a nonprofit group in Boston which provides assistance to young adult survivors of cancer to help the transition to life after treatment.

Lori Fink, Chair of the NYUCI Advisory Board, explained that when she met Mr. Hill in 2010, he believed the worst part of his cancer experience was behind him. "He felt he had his life ahead of him, this had been taken care of, and he wanted to be an upstanding citizen and make a difference," Mrs. Fink noted.

William L. Carroll, MD, NYUCI Director, added, "Jeremy was a dedicated partner in shaping our Board and a generous, unflagging supporter of our work. We extend our deepest sympathies to his family." ■

How You Can Help

Donations to the NYU Cancer Institute can bring us closer to our goal of defeating cancer. Each gift — no matter what its size — furthers our research efforts, enhances our clinical services, and expands our community programs so that we may help more people overcome this illness. You can give online at www.NYUCI.org, where you can:

- Create your own personal fundraising page
- Make an unrestricted gift
- Direct your gift to fund a particular area
- Honor your doctor, nurse, or loved one

Contributions can also be sent to:
NYU Cancer Institute
NYU Langone Medical Center
Office of Development
One Park Avenue, 17th Floor
New York, NY 10016

For more information, please contact Margo Bloom at 212-404-3638 (margo.bloom@nyumc.org)



events calendar

Registration is required for all events, and seating may be limited.

Please call 212-263-2266 or e-mail NYUCIcommunityprograms@nyumc.org for more information and to register, unless otherwise noted.

FOLLOWING THE SEASONS FOR OPTIMAL HEALTH

Nourishing life as it unfolds is one of the basic beliefs of Chinese medicine. During this series of lectures, you will learn simple and effective ways to take care of and nurture yourself.

Thursday, September 13, 6:00 P.M. – 7:30 P.M., Location A*

Topic: Earth Element: Late Summer

Description: The Earth element gives us the capacity to digest and understand, enabling us to transform our intentions into actions. It also allows us to give, receive, and nourish on physical, emotional, and spiritual levels. In this lecture, you will learn to recognize the Earth element in you, recognize when it is out of balance, and identify the types of foods to include during this season to nourish the Earth in you.

Tuesday, October 23, 6:00 P.M. – 7:30 P.M., Location A*

Topic: Metal Element: Fall/Autumn

Description: The Metal element empowers us to take in and hold on to what is useful, and to let go of what no longer serves us. It gives us the capacity to renew and grow, to grieve, and to move on, as well as to receive the acknowledgment and recognition we need to feel complete. In this lecture, you will learn to recognize the Metal element in you, recognize when it is out of balance, and identify the types of foods that will nourish and balance your Metal element.

Presenter: Ooi-Thye Chong, RN, MPH, LAc

CANCER RISK, CANCER SCREENING, AND CONTROVERSY

In recognition of Prostate Cancer Awareness Month

Thursday, September 27, 6:00 P.M. – 7:30 P.M., Location B*

Description: The U.S. Preventive Services Task Force recently released an updated statement on PSA (prostate-specific antigen) testing for men, recommending that healthy men avoid getting regular PSA tests. Our doctors will provide their perspective on the recent controversies surrounding prostate cancer screening and explain why views and beliefs are changing about the validity of screening.

Presenter: Samir Taneja, MD

LYMPHEDEMA EDUCATION SERIES

Wednesday, October 3, 6:00 P.M. – 7:00 P.M., Location C*

Topic: Breast Cancer and Lymphedema

Description: This program is designed to help individuals and their families better understand lymphedema through education.

Presenters: Deborah Axelrod, MD, FACS, and Amber Guth, MD

HEALTHY WOMEN: MAKING THE MOST OF EVERY DAY

In recognition of Breast Cancer Awareness Month

Wednesday, October 17, 12:00 P.M. – 1:30 P.M., Location D*

Description: It is important for women to take charge of their breast health. This session will provide an update on prevention, detection, and screening, addressing the importance of family history, different screening options available, and why mammography is still the gold standard.

Presenters: Monique Hartley-Brown, MD, and Kathie-Ann Joseph, MD

To reserve your seat, call 718-963-8640. Lunch will be provided after the program. This event is presented in collaboration with the Office of Business Affairs

TREATMENT AND SURVIVORSHIP IN WOMEN UNDER 40

In recognition of Breast Cancer Awareness Month

Thursday, October 18, 5:30 P.M. – 7:30 P.M., Location A*

Description: Join us for a lecture focusing on the concerns of younger women with breast cancer. Topics include risk assessment and screening, fertility preservation and childbearing, unique challenges in survivorship, and special treatment considerations for younger women.

Presenters: Nicole Noyes, MD; Ruth Oratz, MD; Freya Schnabel, MD
Moderator: Deborah Axelrod, MD, FACS

Supported by The Arlene and Arnold Goldstein Breast Cancer Educational Program

*Locations:

A: NYU Langone Medical Center, 550 First Avenue, Alumni Hall A

B: NYU Langone Medical Center, 550 First Avenue, Alumni Hall B

C: NYU Clinical Cancer Center, 160 East 34th Street, Room 126

D: Woodhull Medical Center, 760 Broadway, Conference Room #1, Brooklyn, NY 11206

The NYU Cancer Institute helps advance the care of patients with the most common types of cancer and blood disorders, including those of the:

- **Breast**
- **Gynecologic Cancers**
- **Gastrointestinal Tract**
- **Genitourinary System (such as prostate cancer)**
- **Neuro-Oncology (including brain cancer)**
- **Lung**
- **Head and Neck**
- **Melanoma**
- **Hematologic Cancers and other blood disorders**
- **Sarcoma**
- **Pediatric Cancers**

NYU Clinical Cancer Center
160 East 34th Street
New York, NY 10016

As the principal outpatient facility of the NCI-designated NYU Cancer Institute, the NYU Clinical Cancer Center serves as home base for our patients and their caregivers. The center and its multidisciplinary team of experts provide convenient access to the latest treatment options and clinical trials, along with a variety of programs in cancer prevention, screening, diagnostics, genetic counseling, and supportive services.

IMPORTANT PHONE NUMBERS

New Patient Physician Referral Line	212-731-5000
Clinical Trials Information	212-263-6485
Screening Mammography and/or Related Procedures	212-731-5002
NYU Clinical Cancer Center Support Group Information Line	212-731-5480
100 Women in Hedge Funds National Ovarian Cancer Early Detection Program	212-731-5345
Lucille Roberts Wellness Boutique managed by Underneath It All	212-731-5198
Stephen D. Hassenfeld Children's Center for Cancer and Blood Disorders	212-263-8400

Speakers Bureau & Community Outreach Programs	212-263-6342
NYULMC Office of Communications Media Inquiries	212-404-3555
Office of Development/Donations	212-404-3640
NYUCI Office of the Director	212-263-3276

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